

**CASCADE COUNTY EMERGENCY MEDICAL SERVICES**

Prehospital Care Report | Unit R-7 | Shift A

**AMBULANCE RUN REPORT**

**Patient Name** Voss, Renata A.  
**Date of Birth** 07/22/1990 (Age 34)  
**Sex** Female  
**Incident Date / Time** 05/03/2025 11:14  
**Dispatch / On-Scene / Transport** 11:16 / 11:24 / 11:41  
**Receiving Facility** Cascade County Regional Medical Center, ED  
**Run Number** EMS-2025-01772

**DISPATCH INFORMATION**

Unit dispatched Priority 2 to Meridian Grocery Outfitters, 620 Larkspur Boulevard, for a report of a customer fall with a possible arm injury. Store management on scene prior to arrival, wet-floor caution sign reportedly present but knocked over.

**SCENE FINDINGS**

On arrival crew found a 34-year-old female seated on a chair in the store's front office, holding her right arm against her body. Patient stated she slipped on a wet section of tile near the produce section and fell, landing on an outstretched right hand and right shoulder. No loss of consciousness. Store manager provided an incident report number (ML-2025-0614).

**PATIENT ASSESSMENT**

Patient is alert and oriented x4, GCS 15. Complains of immediate onset right wrist pain rated 8/10 with visible swelling and mild deformity of the distal forearm, and right shoulder pain rated 6/10 with limited range of motion. Denies head strike, denies neck pain, denies chest pain, denies shortness of breath, denies loss of consciousness. Skin warm and dry. Radial pulse present and strong on the right, capillary refill less than 2 seconds in all digits, sensation intact to light touch in the right hand.

Right wrist splinted in position found with a padded rigid splint. Right arm placed in a sling for comfort. No other injuries noted on rapid trauma assessment.

**VITAL SIGNS**

|       |                                                 |
|-------|-------------------------------------------------|
| 11:26 | BP 138/84, HR 98, RR 18, SpO2 99% RA, Pain 8/10 |
| 11:38 | BP 132/80, HR 90, RR 16, SpO2 99% RA, Pain 6/10 |

**TREATMENT RENDERED**

- Right wrist splinted, right arm placed in sling
- Ice pack applied to right wrist
- IV access deferred, patient stable
- Oxygen not required, room air maintained

**TRANSPORT**

Transported non-emergent to Cascade County Regional Medical Center. No change in patient condition en route. Report given to ED triage RN at 11:47. Care transferred without incident.

Crew: D. Ferreira, EMT-P (Lead) / M. Aldous, EMT-B. Report electronically signed 05/03/2025 13:05.

**CASCADE COUNTY REGIONAL MEDICAL CENTER**  
Department of Emergency Medicine | 1180 Foothill Drive  
**EMERGENCY DEPARTMENT TRIAGE & NURSING RECORD**

**Patient** Voss, Renata A.  
**MRN** CCR-509214  
**Account / Encounter** ED-25-0503-231  
**Arrival** 05/03/2025 11:47 via EMS  
**Triage Acuity** ESI Level 3  
**Allergies** PENICILLIN (hives, facial swelling, documented by patient); NKA otherwise

**TRIAGE NOTE (11:50)**  
Patient slipped on a wet floor at a grocery store approximately 30 minutes ago, landing on outstretched right hand. Reports right wrist pain 8/10 with visible deformity, and right shoulder pain 6/10. Denies head strike, denies LOC. Arrived splinted per EMS. Pain rated 8/10 at triage.

**NURSING ASSESSMENT**  
Alert, oriented, cooperative, visibly uncomfortable holding right arm guarded against torso. Neurovascular intact right upper extremity distal to injury: radial pulse 2+, capillary refill less than 2 seconds, sensation intact to light touch in median, ulnar, and radial distributions. Visible swelling and dorsal angulation of the distal right forearm. Skin intact, no open wound.

|                                                        |                                                    |
|--------------------------------------------------------|----------------------------------------------------|
| <b><u>INTAKE QUESTIONNAIRE (patient-completed)</u></b> |                                                    |
| Prior fractures or orthopedic injury?                  | NO                                                 |
| Prior shoulder or wrist treatment?                     | NO                                                 |
| Currently working?                                     | YES - dental hygienist, full duty                  |
| Current medications?                                   | Oral contraceptive. Occasional acetaminophen.      |
| Tobacco / Alcohol                                      | Never smoker. Social alcohol, 1-2 drinks per week. |

|                                       |                                                   |
|---------------------------------------|---------------------------------------------------|
| <b><u>VITAL SIGNS - ED COURSE</u></b> |                                                   |
| 11:52                                 | BP 136/82 HR 94 RR 18 T 98.0F SpO2 99% Pain 8/10  |
| 12:30                                 | BP 128/78 HR 84 RR 16 T 98.2F SpO2 99% Pain 5/10  |
| 13:40                                 | BP 124/76 HR 78 RR 16 T 98.1F SpO2 100% Pain 3/10 |

**MEDICATIONS ADMINISTERED IN ED**  
– 12:05 Fentanyl 50 mcg IV x1 - given by T. Okonjo, RN  
– 12:10 Ibuprofen 600 mg PO x1 - given by T. Okonjo, RN

**NURSING DISPOSITION**  
Right forearm placed in a sugar-tong splint following physician reduction and post-reduction imaging. Patient tolerated splinting well. Discharge instructions reviewed, patient verbalized understanding. Departed ambulatory with a coworker at 14:20.

*Electronically signed: T. Okonjo, RN | 05/03/2025 14:25*

## **CASCADE COUNTY REGIONAL MEDICAL CENTER**

Department of Emergency Medicine

### **EMERGENCY PHYSICIAN NOTE**

**Patient** Voss, Renata A. MRN CCR-509214

**Date of Service** 05/03/2025

**Attending** J. Marchetti, MD, FACEP

### **CHIEF COMPLAINT**

Right wrist deformity and right shoulder pain following a fall.

### **HISTORY OF PRESENT ILLNESS**

The patient is a 34-year-old female with no significant past medical history who presents by EMS after slipping on a wet floor at a grocery store earlier today. She fell forward onto an outstretched right hand and struck her right shoulder against a display shelf on the way down. She denies striking her head, denies loss of consciousness, and denies neck pain.

She describes immediate onset of severe right wrist pain with visible deformity, and moderate right shoulder pain with a sensation of the shoulder 'catching' on overhead motion. She denies numbness in the fingers, denies bilateral symptoms, and denies any prior injury to either extremity.

### **PAST MEDICAL HISTORY**

Unremarkable per patient report. No prior surgeries. Oral contraceptive only.

### **ALLERGIES**

Penicillin - hives and facial swelling.

### **PHYSICAL EXAMINATION**

General: Well-developed female, uncomfortable but in no acute distress.

HEENT: Normocephalic, atraumatic.

Right upper extremity: Dorsal angulation and swelling over the distal radius, consistent with a 'dinner fork' deformity. Skin intact, no open wound. Radial pulse 2+, capillary refill less than 2 seconds. Sensation intact median, ulnar, radial nerve distributions. Finger flexion and extension intact though limited by pain.

Right shoulder: Tenderness over the anterior and lateral aspect. Painful arc with abduction between 60 and 120 degrees. Positive empty can test, limited by pain. No obvious deformity, no step-off at the acromioclavicular joint. Distal neurovascular exam intact.

### **MEDICAL DECISION MAKING**

Given visible deformity of the distal forearm, wrist and forearm radiographs were obtained, confirming a displaced distal radius fracture. Closed reduction was performed under procedural sedation with fentanyl. Post-reduction imaging demonstrated improved alignment. Shoulder radiographs were obtained given persistent pain and limited range of motion; no fracture or dislocation identified. Clinical findings are suggestive of a possible rotator cuff strain versus partial tear, for which outpatient MRI may be warranted if symptoms persist.

### **DIAGNOSES**

- Displaced fracture, distal right radius, closed, initial encounter (S52.531A)
- Right shoulder strain/sprain, initial encounter (S43.421A)
- Fall on same level, initial encounter (W01.10XA)

*Electronically signed: Julia Marchetti, MD | 05/03/2025 14:10*

## CASCADE COUNTY REGIONAL MEDICAL CENTER

Department of Radiology

### DIAGNOSTIC IMAGING REPORTS

**Patient** Voss, Renata A. MRN CCR-509214

**Date of Service** 05/03/2025

#### **EXAM 1: RADIOGRAPHS RIGHT WRIST/FOREARM, PRE- AND POST-REDUCTION**

Accession: XR-25-060217 | Performed 12:00 and 12:40 | CPT 73090

INDICATION: Fall onto outstretched hand with visible deformity.

FINDINGS: Pre-reduction images demonstrate a displaced, dorsally angulated fracture of the distal radial metaphysis with mild dorsal comminution, consistent with a Colles-type fracture. The distal ulna is intact. No intra-articular extension identified. Post-reduction images obtained following closed reduction show improved alignment with restoration of radial length and correction of dorsal angulation. Sugar-tong splint in place.

#### IMPRESSION:

- Displaced dorsally angulated distal radius fracture, improved alignment post-reduction.
- No evidence of distal ulna fracture.

*Reported by: F. Adeyemi, MD | Signed 05/03/2025 13:15*

#### **EXAM 2: RADIOGRAPHS RIGHT SHOULDER, 3 VIEWS**

Accession: XR-25-060224 | Performed 12:55 | CPT 73030

INDICATION: Right shoulder pain following fall.

FINDINGS: Glenohumeral joint is congruent, no dislocation. No acute fracture of the proximal humerus, clavicle, or scapula. Mild narrowing of the subacromial space. Acromioclavicular joint unremarkable. Soft tissues unremarkable.

#### IMPRESSION:

- No acute fracture or dislocation of the right shoulder.
- Mild subacromial narrowing; clinical correlation and MRI recommended if symptoms persist.

*Reported by: F. Adeyemi, MD | Signed 05/03/2025 13:40*

## CASCADE COUNTY REGIONAL MEDICAL CENTER

Department of Emergency Medicine

### ED DISCHARGE SUMMARY & PRESCRIPTION RECORD

**Patient** Voss, Renata A. MRN CCR-509214

**Discharge Date / Time** 05/03/2025 14:20

**Disposition** Home, ambulatory, accompanied by coworker

**Condition at Discharge** Stable, improved

#### DISCHARGE DIAGNOSES

Displaced distal radius fracture, status post closed reduction; right shoulder strain; fall injury.

#### PRESCRIPTIONS ISSUED

- 1. Ibuprofen 600 mg PO TID with food, #30, no refills
- 2. Cyclobenzaprine 5 mg PO TID PRN muscle spasm, #21, no refills
- 3. Hydrocodone/Acetaminophen 5/325 mg, 1 tab PO q6h PRN severe pain, #15, no refills

#### DISCHARGE INSTRUCTIONS

- Keep sugar-tong splint clean and dry. Do not remove.
- Elevate right arm above heart level as much as possible for 48 hours to reduce swelling.
- Ice to wrist and shoulder 20 minutes every 2-3 hours for the first 48 hours.
- Work status: OFF WORK 05/04/2025 through 05/09/2025. Follow up with orthopedic hand specialist within 5-7 days for definitive fracture management.
- Follow up with primary care physician if shoulder pain does not improve within two weeks.

#### RETURN PRECAUTIONS - RETURN IMMEDIATELY IF

- Fingers become pale, blue, cold, or numb
- Severe unrelenting pain not controlled by medication
- Signs of infection: fever, redness, or drainage

Patient verbalized understanding of all instructions, prescriptions, and return precautions. All questions answered. Written materials provided.

*Electronically signed: Julia Marchetti, MD | 05/03/2025 14:22*

*Co-signed: T. Okonjo, RN | 05/03/2025 14:25*

**BRIARWOOD FAMILY HEALTH ASSOCIATES**

Renata Voss - Established Patient | 240 Larch Street, Suite 4

**OFFICE VISIT NOTE - PRIMARY CARE**

**Patient** Voss, Renata A. DOB 07/22/1990

**Date of Service** 05/12/2025

**Provider** N. Castellano, MD

**Visit Type** Follow-up, post fall injury

**SUBJECTIVE**

Patient returns 9 days after a fall with a distal radius fracture treated with closed reduction and splinting, and an associated right shoulder strain. She reports wrist pain has improved from 8/10 to roughly 3/10 in the splint. Right shoulder pain persists at 5-6/10, worse with overhead reaching and at night. She reports difficulty sleeping on the right side.

She took the hydrocodone/acetaminophen for three days and stopped due to nausea; ondansetron was not needed as symptoms resolved with discontinuation. She continues ibuprofen and cyclobenzaprine with partial benefit. She has not returned to work; her employer requires two-handed instrument use for dental hygiene duties that she cannot currently perform.

Of note, on further questioning the patient recalls a minor right shoulder strain approximately four years ago from a recreational volleyball game, treated with rest and over-the-counter medication, fully resolved with no imaging performed at that time.

**OBJECTIVE**

|                       |                                                                                                                                                                                                    |
|-----------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Vitals</b>         | BP 118/74, HR 72, T 98.2F, Wt 148 lb, Ht 5'6", BMI 23.9                                                                                                                                            |
| <b>Right wrist</b>    | Splint in place, intact, no drainage. Fingers well perfused, sensation intact.                                                                                                                     |
| <b>Right shoulder</b> | Tenderness over the anterior joint line and greater tuberosity. Painful arc 60-120 degrees. Positive empty can test. Positive drop-arm test, weak. External rotation strength 4/5 limited by pain. |
| <b>Neuro</b>          | Distal sensation intact bilaterally. Grip strength not tested due to splint.                                                                                                                       |

**ASSESSMENT**

- 1. Distal radius fracture, healing appropriately in splint, per orthopedic plan.
- 2. Right shoulder pain with positive impingement and weakness signs, concerning for rotator cuff tear, not improving at 9 days.
- 3. Nausea, resolved, attributed to hydrocodone/acetaminophen.

**PLAN**

- Discontinue hydrocodone/acetaminophen given nausea; continue ibuprofen and cyclobenzaprine.
- Referral to orthopedic hand surgery for fracture follow-up.
- Referral to orthopedic shoulder specialist given exam findings concerning for rotator cuff injury.
- MRI right shoulder without contrast ordered given persistent weakness and positive drop-arm test.
- Work note provided: off work through 05/23/2025, reassess.

*Electronically signed: Nadia Castellano, MD | 05/12/2025 15:40*

**TRAILHEAD HAND & UPPER EXTREMITY THERAPY**

Outpatient Occupational Therapy | Clinic Site 1

**OCCUPATIONAL THERAPY INITIAL EVALUATION**

**Patient** Voss, Renata

**DOB** 07/22/1990

**Date of Evaluation** 05/20/2025

**Referring Provider** N. Castellano, MD

**Therapist** B. Okafor, OTR/L, CHT

**REFERRAL DIAGNOSIS**

Distal radius fracture, right, status post closed reduction; right shoulder strain with suspected rotator cuff involvement. Frequency 2x/week for 6 weeks.

**SUBJECTIVE**

Patient reports right wrist pain averaging 3/10, aggravated by gripping and rotation. Right shoulder pain averaging 5/10, aggravated by reaching overhead or behind the back, easing with rest. Sleep disrupted by shoulder pain, waking 2-3 times nightly. Reports difficulty with occupational tasks requiring fine motor control and sustained grip.

**OBJECTIVE FINDINGS**

|                                    |                                                                                 |
|------------------------------------|---------------------------------------------------------------------------------|
| <b>Right wrist AROM</b>            | Flexion 30 deg, Extension 25 deg, limited by residual splint stiffness and pain |
| <b>Right shoulder AROM</b>         | Flexion 110 deg, Abduction 95 deg, External rotation 30 deg                     |
| <b>Grip strength (dynamometer)</b> | Right 14 lb / Left 62 lb                                                        |
| <b>Palpation</b>                   | Tenderness over supraspinatus insertion and bicipital groove                    |
| <b>Special tests</b>               | Positive empty can, positive Neer impingement sign, positive drop-arm (weak)    |

**OUTCOME MEASURES**

DASH (Disabilities of the Arm, Shoulder and Hand) score: 61% (severe disability). Numeric Pain Rating: 5/10 current, 8/10 worst, 2/10 best.

**ASSESSMENT**

Patient presents with post-fracture wrist stiffness and signs consistent with a right rotator cuff tear, with significant functional limitation in grip strength and shoulder elevation. Good rehabilitation potential for the wrist; shoulder prognosis pending MRI findings.

**PLAN OF CARE / GOALS (6 weeks)**

- Restore wrist flexion/extension to within 10 degrees of the uninvolved side
- Improve grip strength to 40 lb or greater
- Reduce worst shoulder pain from 8/10 to 4/10
- Improve DASH score to below 30%
- Return to modified duty work

Interventions: manual therapy, scar and edema management, progressive wrist ROM and strengthening, scapular stabilization exercise, modalities as indicated.

*Electronically signed: Blessing Okafor, OTR/L, CHT | 05/20/2025*

## SUMMIT DIAGNOSTIC IMAGING

Advanced Diagnostic Imaging | 1.5T MRI Suite

### MRI RIGHT SHOULDER WITHOUT CONTRAST

**Patient** Voss, Renata A.

**DOB** 07/22/1990

**Date of Exam** 06/02/2025

**Accession** MR-25-33481 CPT 73221

**Referring** N. Castellano, MD

### CLINICAL HISTORY

34-year-old female, fall on outstretched hand 05/03/2025 with associated shoulder strain. Persistent weakness, positive drop-arm sign. Evaluate for rotator cuff tear.

### TECHNIQUE

Multiplanar, multisequence imaging of the right shoulder performed without intravenous contrast. Axial, sagittal oblique, and coronal oblique T1 and fluid-sensitive sequences obtained.

### FINDINGS

Rotator cuff: There is a full-thickness tear of the distal supraspinatus tendon measuring approximately 1.4 cm in the anteroposterior dimension, with mild retraction of the tendon edge to the level of the humeral head. Mild tendinosis of the infraspinatus without discrete tear. Subscapularis and teres minor intact.

Subacromial-subdeltoid bursa: Mild bursal fluid, consistent with bursitis.

Biceps tendon: Mild tenosynovitis of the long head of the biceps tendon within the bicipital groove.

Labrum: No labral tear identified. Glenohumeral joint congruent.

Osseous structures: No fracture, no marrow edema. Mild acromial spurring, type II acromion morphology.

AC joint: Mild degenerative change, no significant osteoarthritis.

### IMPRESSION

- 1. Full-thickness tear of the distal supraspinatus tendon, approximately 1.4 cm, with mild retraction.
- 2. Mild infraspinatus tendinosis without tear.
- 3. Mild subacromial-subdeltoid bursitis and biceps tenosynovitis, likely reactive.
- 4. No labral tear or fracture. Findings correlate with the reported traumatic mechanism; no significant degenerative tear morphology to suggest a chronic pre-existing process.

*Reported by: C. Ibarra, MD | Electronically signed 06/02/2025 16:20*



## STONEBRIDGE ORTHOPEDIC & SPORTS MEDICINE

Shoulder and Upper Extremity Surgery

### ORTHOPEDIC SHOULDER CONSULTATION

**Patient** Voss, Renata A. DOB 07/22/1990

**Date of Service** 06/10/2025

**Provider** E. Whitcombe, MD, Orthopedic Surgery, Shoulder Specialist

**Referral Source** N. Castellano, MD

### HISTORY

This is a 34-year-old right-hand-dominant female dental hygienist referred for evaluation of a right rotator cuff tear following a fall at work approximately five weeks ago. She was evaluated in the emergency department on the date of injury with a concurrent distal radius fracture, treated with closed reduction and splinting, now healing well. Shoulder symptoms have persisted despite NSAIDs, a muscle relaxant, and five weeks of occupational/physical therapy.

She describes shoulder pain of 5/10 at rest and 8/10 with overhead activity, with associated weakness lifting the arm and an inability to sleep on the right side. She denies numbness or tingling into the hand, denies neck pain, and denies prior shoulder surgery.

She remains off work. She reports her occupation requires sustained overhead and forward reaching that she is currently unable to perform.

### REVIEW OF PRIOR RECORDS

I have personally reviewed the ED records of 05/03/2025, the wrist and shoulder radiographs of the same date, and the MRI right shoulder of 06/02/2025. Occupational therapy notes reviewed.

### EXAMINATION

Right shoulder: Visible mild deltoid atrophy compared to the left. Active abduction limited to 100 degrees, passive abduction to 150 degrees. Positive drop-arm sign. Positive empty can test with pain and weakness 4-/5. External rotation strength 4/5. Positive Neer and Hawkins impingement signs. AC joint non-tender. No instability on apprehension testing.

Right wrist: Well-healed position, no longer splinted. Wrist flexion/extension approximately 80% of the left side. Grip strength improving per therapy notes.

### ASSESSMENT

Full-thickness right supraspinatus tear, traumatic in etiology, correlating with the reported fall and with imaging. Failed conservative management including activity modification, medication, and structured occupational therapy over approximately five weeks.

### PLAN

- 1. Discussed non-operative versus operative management. Given the patient's age, full-thickness tear with early retraction, and occupational demands, recommend arthroscopic rotator cuff repair.
- 2. Pre-operative labs and medical clearance ordered.
- 3. Continue occupational therapy for the wrist in the interim; hold aggressive shoulder strengthening pending surgery.
- 4. Surgery scheduled for arthroscopic right rotator cuff repair with possible subacromial decompression and biceps tenodesis depending on intraoperative findings.
- 5. Work status: remains off work pending surgery.

*Electronically signed: Elliot Whitcombe, MD | 06/10/2025*

## OFFICE OF THE RECORDS CUSTODIAN

Certification and Production Log

### RECORDS PRODUCTION CERTIFICATION

**Matter** Voss v. Meridian Grocery Outfitters LLC

**Claim Number** PI-2025-4432

**Records Requested By** Requesting Party (records request dated 06/05/2025)

**Date of Production** 06/18/2025

**Total Pages Produced** 10

### CERTIFICATION

The undersigned certifies that the accompanying documents are true and correct copies of records maintained in the ordinary course of business, produced in response to the request referenced above, and that the records were made at or near the time of the events described by persons with knowledge, or from information transmitted by such persons.

### PRODUCTION LOG - SOURCES

|                                                    |         |
|----------------------------------------------------|---------|
| Cascade County EMS                                 | 1 page  |
| Cascade County Regional Medical Center - ED        | 3 pages |
| Cascade County Regional Medical Center - Radiology | 1 page  |
| Briarwood Family Health Associates                 | 1 page  |
| Trailhead Hand & Upper Extremity Therapy           | 1 page  |
| Summit Diagnostic Imaging                          | 1 page  |
| Stonebridge Orthopedic & Sports Medicine           | 1 page  |
| This certification                                 | 1 page  |

### ITEMS REQUESTED BUT NOT PRODUCED

- Pre-operative clearance and surgical records for the scheduled arthroscopic rotator cuff repair. Surgery had not yet occurred as of the date of this production.
- Store incident report (ML-2025-0614) referenced in the EMS run report. Requested from Meridian Grocery Outfitters 06/05/2025, no response received.
- Pharmacy dispensing history. Requested from patient-identified pharmacy 06/05/2025, no response received.
- Employer wage and work-status records. Requested from employer 06/05/2025, no response received.

*Certified by: Office of the Records Custodian | 06/18/2025*